CAREER GLITCH · SHORT · SIMPLE · NO BORING PARTS

RESPIRATORY THERAPIST IN THE AI ERA

When a patient can't breathe, this is the specialist who gets called — not eventually, now.

★ TL;DR — Around $75K median after a 1–2 year accredited program, board exams and a state license. The catch: ICU intensity is the job — codes, vents and nights. ★

THE CAREER GLITCH TAKE

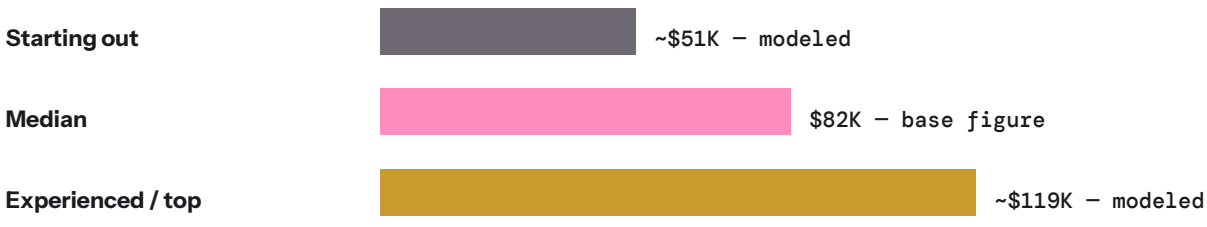
OUR TAKE: YES

MEDIAN ~$82K · TRAINING 1-2 years · AI RISK LOW · STATE LICENSE

- A focused specialty: lungs, airways, ventilators — deep expertise in one critical system.
- The state license plus board credentials make this a protected scope — hospitals can't substitute around you.
- AI tunes ventilators and predicts deterioration, but airway emergencies are hands-on and immediate.

The site's take, built from this profession's numbers in the Career Glitch base.

THE MONEY — LOOK AT THE BARS



The Career Glitch base figure for Respiratory Therapist is a ~$82K median (compiled from U.S. Bureau of Labor Statistics, Occupational Outlook Handbook ranges). The starting and top bars are modeled around that median — orientation, not a promise. Pay swings hard by state, employer and experience: check live postings in your state before planning around any number.

WHAT DOES A RESPIRATORY THERAPIST ACTUALLY DO?

✓ Set up and manage ventilators and oxygen therapy in ICU, ER and wards

- ✓ Run breathing treatments, draw arterial blood gases, assess lung function
- ✓ Respond to codes and rapid responses as the airway expert
- ✓ Wean patients off ventilation and teach chronic lung patients their routines

Think: the pilot of the breathing system — routine flights, and emergencies where you're the only expert in the room.

WHAT AI ALREADY TOOK (AND WHAT IT CAN'T)

THE ROBOT	THE HUMAN
Ventilator automation suggests settings and liberation timing; monitoring AI flags deterioration patterns early. Documentation and routine checks keep automating.	Intubation support, airway rescue, reading a struggling patient in seconds. When breathing fails, the response is physical, immediate and licensed.

⚠ **You will run toward the worst minutes of people's lives, on night shift, for years. The adrenaline is the appeal and the cost — price it honestly.**

THE VIBE — IS THIS YOUR LIFE?

THE JOB IS	THE JOB IS NOT
✓ Respected specialty with a clear, protected scope ✓ ICU variety: no two shifts breathe the same ✓ Strong hospital demand anchors the pay	✗ Codes, deaths and family grief are regular ✗ Nights and weekends, especially early career ✗ Physically and emotionally heavy rotations

Good fit? Calm in true emergencies, technical with machines, and steady around critical illness.
Bad fit? You want low-stakes days or can't do nights — the ICU doesn't offer either.

THE FORK — PICK ONE DOOR

DOOR A — THE ACCREDITED RT PROGRAM	DOOR B — HOSPITAL-AFFILIATED PROGRAMS
Apply to an accredited respiratory therapy program (typically an associate degree, 1–2 years) with ICU clinical rotations. Graduation opens the board exams and then your state license — both mandatory before you practice.	Some hospital systems host or partner with RT programs, with clinicals inside their own ICUs. Same boards and license at the end; the advantage is hiring familiarity when you graduate.

Smart move: In clinicals, chase ventilator and ICU rotations over floor assignments — new-grad hiring managers ask specifically who managed vents as a student.
Where you land after: Respiratory therapist → ICU/transport specialist, ECMO teams, pulmonary function labs, department leadership.

YOUR ROADMAP — 6 STEPS

1	2	3	4	5	6
MAP ACCREDITED	CLEAR PREREQUISIT	APPLY AND	DO THE	BOARDS, THEN	TAKE THE

STEP 1 — Map accredited programs

FREE · TODAY

- List accredited RT programs and their clinical sites — ICU access matters.
- Check your state's licensing requirements so the path is clear from day one.

STEP 2 — Clear prerequisites

SEMESTER

- Anatomy & physiology, chemistry, math — admissions weigh these.
- Shadow an RT through an ICU shift if a hospital allows it.

STEP 3 — Apply and enroll

APPLICATION SEASON

- Apply to every accredited program you can attend.
- Seats and clinical slots are limited; early and complete files win.

STEP 4 — Do the program

1–2 YEARS

- Cardiopulmonary science, ventilator management, pharmacology, critical care.
- Clinical rotations: treat ICU time as the main event.

STEP 5 — Boards, then the state license

EXAM FEES · AFTER GRADUATION

- Pass the national board exams (TMC, and the clinical simulation for the RRT credential).
- Apply for your state respiratory care license — you cannot practice without it.

STEP 6 — Take the first RT job

JOB HUNT · 2–6 WEEKS

- Hospitals hire from their clinical programs first.
- First year: volunteer for codes and vent patients — competence compounds fast.

FIRST 90 DAYS — JUST DO THIS

- □ Today: list accredited programs and read your state's RT license requirements.
- □ Days 1–30: start prerequisites; arrange an ICU shadowing experience.
- □ Days 31–60: prerequisites running; applications drafted.
- □ Days 61–90: submit applications to all reachable programs.

 Day 90: you're in the pipeline toward boards and a state license — the ICU runway is set.

NEXT — MAKE IT PERSONAL

This roadmap is the same for everyone. Your life is not.

- Not sure this career fits YOU? Take the Big Career Test — 50 questions, your top-3 matches and one honest “probably NOT you”. → careerglitch.com/#test
- Already sure? Turn AI into Your Personal Career Mentor — a 2,300-word system that interviews you first, checks your hours, budget and deadline honestly, and builds YOUR plan into a Notion dashboard. → careerglitch.com/#prompt

★ **WHEN BREATHING FAILS, YOU DON'T.** ★

CAREER GLITCH · Figures: Career Glitch profession base (medians and training ranges compiled from U.S. Bureau of Labor Statistics, Occupational Outlook Handbook ranges). Modeled bars are estimates around the base median. License and certification rules vary by state — verify with your state board before paying for any program.